3.14 Mechanical Ventilation

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • *No specific complaint* **Objective** • ET tube in situ • Secretions in the tube • High-pressure alarm going off Coarse crackles	**ACTUAL** **Ineffective airway clearance** related to presence of artificial airway and inability to cough as evidenced by secretions and high airway pressure.	Airway stays patent and breath sounds are clear.	• Listen to the lungs every 2–4 hours. • Suction when needed with sterile technique; pre-oxygenate first. • Every shift, check the tube position (marking at the lips) and the cuff pressure. • Answer ventilator alarms straight away. • Humidify the gas he breathes in.	Breath sounds clear, pressures normal: **goal met**.
Subjective • Nods but cannot talk **Objective** • Intubated • Gestures in frustration	**ACTUAL** **Impaired verbal communication** related to endotracheal tube as evidenced by inability to speak.	Gets his needs across by other means.	• Ask questions he can answer yes or no. • Give him a writing board or a picture chart. • Explain every procedure to him.	Communicates his needs: **goal met**.
Subjective • *No specific complaint* **Objective** • Intubated for more than 48 hours • Lying flat • Sedated	**RISK** **Risk for infection** as evidenced by risk factors: invasive airway and impaired cough (ventilator-associated pneumonia).	No ventilator-associated pneumonia.	• Head of the bed at 30–45 degrees. • Oral care with antiseptic, following protocol. • Every day, check whether he is ready for a sedation break and weaning. • Drain condensate away from the patient. • Use closed suction systems as policy says.	No new infiltrates, no fever: **goal met**.